RxDx
Be At Your Best. Everyday.

Clinical Laboratory
Telerad RxDx Healthcare Private Limited
Second Floor Plot No 7G, Kundalahalli Main Road, Whitefield, Bengaluru – 560048
901 409 1111 901 996 1402 (Chat with our rep on WhatsApp) rxdx.in | lab@rxdx.in

LABORATORY INVESTIGATION REPORT

Patient Name	: Mr. NIRANJANA DATTA K A	**Age/Sex**	: 47 Year(s)/Male
UHID	: RXML.44153	**Order Date**	: 15/03/2025 09:02
Episode	: OP		
Ref. Doctor	: Health Check - SAMANVAY	**Mobile No**	: 9008255262
		DOB	: 07/08/1977
Address	: . , ,Malleswaram,Karnataka ,0	**Facility**	: RxDx Whitefield

Clinical Pathology

Sample No : 11H0000068 Collection Date : 15/03/25 09:11 Ack Date : 15/03/2025 15:28 Report Date : 15/03/25 17:20

Complete Urine Analysis

Test Name	Result	Unit	Ref. Range
Colour *Sample - Urine*	PALE YELLOW		PALE YELLOW
Volume *Sample - Urine*	30ML		ML
Reaction *Method - Macroscopy , Sample - Urine*	ACIDIC		ACIDIC
Appearance *Sample - Urine*	CLEAR		CLEAR
PH *Method - Reflectance photometry, Sample - Urine*	7.0		5.0 - 8.0
Specific Gravity *Method - Reflectance Photometry, Sample - Urine*	1.015		1.005 - 1.030
Protein *Method - Reflectance Photometry, Sample - Urine*	ABSENT		ABSENT
Glucose *Method - Reflectance Photometry, Sample - Urine*	ABSENT		ABSENT
Bile Salt *Method - Reflectance Photometry, Sample - Urine*	NEGATIVE		NEGATIVE
Bile Pigments *Method - Reflectance Photometry, Sample - Urine*	NEGATIVE		NEGATIVE
Ketonebodies *Method - Reflectance Photometry, Sample - Urine*	NEGATIVE		NEGATIVE
Leucocyte Esterase *Sample - Urine*	NEGATIVE		NEGATIVE
Nitrite *Method - Reflectance Photometry, Sample - Urine*	NEGATIVE		NEGATIVE
Urinobilingen *Method - Photoelectric Colorimetry , Sample - Urine*	NORMAL		NORMAL
Microscopy *Method - Macroscopy , Sample - Urine*	-		
Puscells *Method - Microscopy , Sample - Urine*	3 - 4	/HPF	0 - 5
Epithelial Cells *Method - Microscopy , Sample - Urine*	1 - 2	/HPF	4 - 5
RBC *Method - Microscopy , Sample - Urine*	ABSENT	/HPF	ABSENT
Casts *Method - Microscopy , Sample - Urine*	ABSENT	/HPF	ABSENT
Crystal *Sample - Urine*	ABSENT		ABSENT

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Others *Method - Microscopy , Sample - Urine*	ABSENT	ABSENT

End of Report

Dr.Seema Pavan
MBBS, MD(Pathology),PGDMLE
Consultant Pathologist, KMC N0. 36984

LABORATORY INVESTIGATION REPORT

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Biochemistry

Test Name	Result	Unit	Ref. Range

Sample No : 11H0000068 Collection Date : 15/03/25 09:11 Ack Date : 15/03/2025 15:18 Report Date : 15/03/25 17:20

Serum Calcium

Calcium Serum *Method - Photometric assay using BAPTA, Sample - Serum*	9.9	mg/dl	8.6 - 10

"Interpretation:

Milder degrees of insufficiency is believed to cause reduced efficiency in the utilization of dietary calcium. Vitamin D deficiency causes muscle weakness; in the elderly, the risk of falling has been attributed to the effect of vitamin D on muscle function. Vitamin D deficiency is a common cause of secondary hyperparathyroidism. Elevations of parathyroid hormone levels, especially in elderly vitamin D deficient adults can result in osteomalacia, increased bone turnover, reduced bone mass, and risk of bone fractures. Low 25-hydroxyvitamin D concentrations are also associated with lower bone mineral density. In conjunction with other clinical data, the results may be used as an aid in the assessment of bone metabolism Insufficiency has been linked to diabetes, different forms of cancer, cardiovascular disease, autoimmune diseases, and innate immunity.

Serum calcium levels and hence the body content are controlled by parathyroid hormone (PTH), calcitonin, and vitamin D. An imbalance in any of these modulators leads to alterations of the body and serum calcium levels. Increases in serum PTH or vitamin D are usually associated with hypercalcemia. Increased serum calcium levels may also be observed in multiple myeloma and other neoplastic diseases. Hypocalcemia may be observed e.g. in hypoparathyroidism, nephrosis, and pancreatitis."

Glycated Haemoglobin (Hba1C)

GLYCATED HEMOGLOBIN(HbA1c) *Method - HPLC, Sample - EDTA-HBA1C*	**6.0 ▲ (H)**	%	Normal 4.6 - 5.7% Prediabetes 5.7% - 6.4 % Diabetes 6.5% or Higher
ESTIMATED AVERAGE GLUCOSE *Method - caluclated, Sample - EDTA-HBA1C*	**125.50 ▲ (H)**	mg/dl	< 117

"Interpretation:

HbA1c is a glycated form of hemoglobin formed by the attachment of glucose residues in the blood to the Hb molecules. The level of HbA1c is proportional to the level of glucose in the blood and accepted as an indicator of the mean blood glucose concentration in the preceding 6-8 weeks. It is a long term indicator of diabetic control. Levels should be monitored every 3-4 months.

Serum Creatinine

Serum Creatinine *Method - Kinetic (Jaffe Method), Sample - Serum*	0.80	mg/dl	0.7 - 1.2

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"Interpretation:

Creatinine is measured to primarily assess kidney function over urea. Since its rate of production is constant, the elevation of plasma creatinine is indicative of under–excretion, suggesting kidney impairment. A decreased level of creatinine has no clinical significance."

Blood Urea Nitrogen (BUN)

Blood Urea Nitrogen *Method - Urease - GLDH, Sample - Serum*	7.70	mg/dl	5 - 25

Serum Uric Acid

Serum Uric Acid *Method - Enzymatic Colorimetric Test Uricase, Sample - Serum*	6.30	mg/dl	3.4 - 7

"Interpretation:

Uric acid is an end product of purine metabolism. An increased serum level of uric acid (hyperuricemia) is found in gout (leading to the formation of sodium urate crystals around joints). Other causes of hyperuricemia are renal diseases, starvation, drug abuse, increased alcohol intake, and certain drugs. Hypourecemia is seldom observed and associated with rare hereditary metabolic disorders."

Lipid Profile

Total Cholesterol *Method - Enzymatic Colorimetric Test CHOD - PAP, Sample - Serum*	168.0	mg/dl	Desirable cholesterol level : < 200 Borderline high cholesterol:200-240 High cholesterol : = 240
Triglycerides *Method - Enzymatic, Sample - Serum*	133.0	mg/dl	Normal <200 High =>200
HDL Cholesterol *Method - Direct Enzymatic Colorimetric Test, Sample - Serum*	**38.2 ▼ (L)**	mg/dl	Male No risk : >55 Moderate risk : 35 - 55 High risk :<35
LDL Cholesterol *Method - CALCULATED, Sample - Serum*	**103.20 ▲ (H)**	mg/dl	<100 Good 100- 129 Near optimal 130 - 159 Borderline
VLDL Cholesterol *Method - CALCULATED, Sample - Serum*	26.60	mg/dl	10 - 40
CHOL:HDL RATIO *Method - CALCULATED, Sample - Serum*	4.40	mg/dl	5.0

LFT - Liver Function Test

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Total Bilirubin *Method - Colorimetric Diazo, Sample - Serum*	0.52	mg/dl	Age of newborn: 24 hours :(> 8.0 *mg/dL) 48 hours:(> 13.0* mg/dL) 84 hours:(> 17.0* mg/dL) Children with age = 1 month:(up to 1.0 mg/dL) Adults Males : (up to 1.4mg/dL) Females:(up to 0.9 mg/dL)
Direct Bilirubin *Method - Colorimetric Diazo, Sample - Serum*	0.26	mg/dl	= 0.30 mg/d
Indirect Bilirubin *Method - CALCULATED, Sample - Serum*	0.26	mg/dl	0.001 - 0.4
Total Protein *Method - Biuret(Colorimetric Assay), Sample - Serum*	7.32	g/dl	Adults : 6.6 - 8.7 g/dL Newborn :4.6-7.0 g/dL 1 week :4.4-7.6 g/dL 7 months-1 year :5.1-7.3 g/dL 1-2 years :5.6-7.5 g/dL > 3 years :6.0-8.0 g /dL true
Albumin *Method - Bromocresol Green(BCG), Sample - Serum*	4.53	g/dl	Adults : 3.5 to 5.2 g/dL 0-4 days: 2.8 to 4.4 g/dL 4 days-14 years : 3.8 to 5.4 g/dL 14 yrs to 18 years : 3.2 to 4.5 g/dL
Globulin *Sample - Serum*	2.79	g/dl	1.8 - 3.4
A:G Ratio *Sample - Serum*	1.62	:1	

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LABORATORY INVESTIGATION REPORT

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SGOT/ AST Serum *Method - UV Kinetic, Sample - Serum*	13.2	U/L	Males up to 40 U/L Females up to 32 U/L
SGPT/ ALT Serum *Method - UV Kinetic, Sample - Serum*	10.5	U/L	Males up to 41 U/L Females up to 33 U/L
Alkaline Phosphatase *Method - PNP AMP UV Kinetic, Sample - Serum*	81.9	U/L	Males 40-130 Females 35-105 Children aged 1 day < 250 aged 2-5 days < 231 aged 6 days-6 months < 449 aged 7 months-1 year < 462 aged 1-3 years < 281 aged 4-6 years < 269 aged 7-12 years < 300 aged 13-17 years (f) < 187
Gamma GT *Method - Enzymatic Colorimetric Test (SZASZ), Sample - Serum*	16.8	U/L	Men < 60 U/L Women < 40 U/L
FBS-Fasting Blood Sugar			
FBS-FASTING BLOOD SUGAR *Method - Hexokinase(HK), Sample - FBS-Plasma*	91	mg/dl	Normal 70 -100 Prediabetes 100 - 125 Diabetes 126

"Interpretation:

Fasting Blood Glucose test measures the level of glucose in the blood after an 8 to 12 hour overnight fast. This test is used to screen for prediabetes, diabetes, or gestational diabetes and monitor the efficacy of medications or lifestyle changes for diabetic people."

Haematology

Test Name	Result	Unit	Ref. Range
Haemogram			

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Haemoglobin *Method - Cyanmethemoglobin-Spectrophotometry, Sample - EDTA-CBC+ESR*	14.2	g/dl	13 - 17
RBC Count *Method - Electrical Impedence, Sample - EDTA-CBC+ESR*	5.37	x10^6/ul	4.5 - 5.5
PCV *Method - Calculated, Sample - EDTA-CBC+ESR*	40.1	%	40 - 50
MCV *Method - Calculated, Sample - EDTA-CBC+ESR*	**74.7** ▼ (L)	fl	83 - 101
MCH *Method - Calculated, Sample - EDTA-CBC+ESR*	**26.5** ▼ (L)	pg	27 - 32
MCHC *Method - Calculated, Sample - EDTA-CBC+ESR*	**35.4** ▲ (H)	g/dl	31.5 - 34.5
RDW CV *Method - Electrical Impedence, Sample - EDTA-CBC+ESR*	**14.6** ▲ (H)	%	11.6 - 14
MPV *Method - Calculated, Sample - EDTA-CBC+ESR*	8.7	umol/L	8 - 11
Platelet *Method - Electrical Impedence, Sample - EDTA-CBC+ESR*	291.0	x10^3/ul	150 - 410
ESR *Method - Manual Westegren Method/ Automated Photometry, Sample - EDTA-CBC+ESR*	2	mm/hr	0 - 15
Tc-Total Leukocyte Count (TLC) *Method - Electrical Impedence, Sample - EDTA-CBC+ESR*	6520	cells/cumm	4000 - 10000
Differential Count *Sample - EDTA-CBC+ESR*			
Neutrophils *Method - Flow cytometry principle, Sample - EDTA-CBC+ESR*	64.8	%	40 - 80
Lymphocytes *Method - Flow cytometry principle, Sample - EDTA-CBC+ESR*	27.6	%	20 - 40
Monocytes *Method - Flow cytometry principle, Sample - EDTA-CBC+ESR*	5.1	%	2 - 10
Eosinophils *Method - Flow cytometry principle, Sample - EDTA-CBC+ESR*	2.5	%	1 - 6
Basophils *Method - Flow cytometry principle, Sample - EDTA-CBC+ESR*	**0.0** ▼ (L)	%	1 - 2
Absolute Neutrophil Count *Sample - EDTA-CBC+ESR*	4.23	x10^3/ul	1.6 - 7
Absolute Lymphocyte Count *Sample - EDTA-CBC+ESR*	1.80	cells/cumm	1 - 3
Absolute Monocyte Count *Sample - EDTA-CBC+ESR*	0.33	cells/cumm	0.2 - 0.8
Absolute Eosinophil Count *Method - Flow Cytometry , Sample - EDTA-CBC+ESR*	0.16	cells/cumm	0 - 0.5
Absolute Basophil Count *Sample - EDTA-CBC+ESR*	0.00	cells/cumm	0 - 0.15

All abnormal parameters are rechecked by Pathologists Manually.

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Peripheral Smear

Peripheral Blood Smear (PS)
Method - Lishman stain, Light microscopy, Sample - EDTA Blood

Report on Peripheral Smear

Erythrocytes: Normocytic Hypochromic.

Leukocytes : Normal in count & morphology, and distribution.

Platelets: Adequate

Haemoparasites: Nil

Other's: Nil

Impression : Normocytic Hypochromic Blood Picture

Immunoassays- Hormones

Test Name	Result	Unit	Ref. Range
25-Hydroxy Vitamin D Total			
25 OH Vitamin D *Method - ECLIA-electrochemiluminescence immunoassay, Sample - Serum*	14.6	ng/ml	VITAMIN D DEFICENT [< 20 NG/ML] INSUFFICIENT [20 - 29 NG/ML] SUFFICENT [30 - 100 NG/ML] POTENTIAL TOXICITY [>100 NG/ML]

"Interpretation:

Milder degrees of insufficiency is believed to cause reduced efficiency in the utilization of dietary calcium. Vitamin D deficiency causes muscle weakness; in the elderly, the risk of falling has been attributed to the effect of vitamin D on muscle function. Vitamin D deficiency is a common cause of secondary hyperparathyroidism. Elevations of parathyroid hormone levels, especially in elderly vitamin D deficient adults can result in osteomalacia, increased bone turnover, reduced bone mass and risk of bone fractures. Low 25-hydroxyvitamin D concentrations are also associated with lower bone mineral density. In conjunction with other clinical data, the results may be used as an aid in the assessment of bone metabolism Insufficiency has been linked to diabetes, different forms of cancer, cardiovascular disease, autoimmune diseases and innate immunity."

PSA (Prostatic Specific Antigen Total)

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LABORATORY INVESTIGATION REPORT

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PSA(PROSTATIC SPECIFIC ANTIGEN TOTAL) *Method - ECLIA-electrochemiluminescence immunoassay, Sample - Serum*	0.67	ng/ml	

"Interpretation:

Elevated concentrations of PSA in serum are generally indicative of a pathologic condition of the prostate (prostatitis, benign hyperplasia or carcinoma) As PSA is also present in para-urethral and anal glands, as well as in breast tissue or with breast cancer, low levels of PSA can also be detected in sera from women. PSA may still be detectable even after radical prostatectomy. The main areas in which PSA determinations are employed are the monitoring of progress and efficiency of therapy in patients with prostate carcinoma or receiving hormonal therapy. The steepness of the rate of fall in PSA down to no longer detectable levels following radiotherapy , hormonal therapy, or radical surgical removal of the prostate provides information on the success of therapy. An inflammation or trauma of the prostate (e.g. in cases of urinary retention or following rectal examination, cystoscopy, coloscopy, transurethral biopsy, laser treatment, or ergometry) can lead to PSA elevations of varying duration and magnitude. "

TSH-Thyroid Stimulating Hormone

Thyroid Stimulating Hormone *Method - ECLIA-electrochemiluminescence immunoassay, Sample - Serum*	1.85	µIU/ml	

"Interpretation:

The determination of TSH serves as the initial test in thyroid diagnostics. Even very slight changes in the concentrations of the free thyroid hormones bring about much greater opposite changes in the TSH level. Accordingly, TSH is a very sensitive and specific parameter for assessing thyroid function and is particularly suitable for early detection or exclusion of hyperthyroidism (decreased TSH values) and hypothyroidism (increased TSH values). "

Serology

Test Name	Result	Unit	Ref. Range
C-Reactive Protein			
C-REACTIVE PROTIEN *Method - Immunoturbidimetric, Sample - Serum*	2.83	mg/L	

"Interpretation:

CRP (C-reactive protein) is a cytokine-induced acute phase protein that increases in concentration as a result of inflammation. CRP levels in the body have been used as a marker or indicator of infections and inflammation . The assay of CRP is more sensitive than the erythrocyte sedimentation rate (ESR) and leukocyte count. The CRP levels return to reference ranges more rapidly after the disease has subsided ."

Test Name	Result	Unit	Ref. Range
Hbs Ag (Rapid Card Test)			
HBsAg (Rapid Card Test) *Method - Lateral Flow Chromatography , Sample - Serum*	Non-Reactive		NON REACTIVE

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"Interpretation:

The nonreactive result implies that HBsAg surface antigen has not been detected in the sample. This is only a screening test. Additional follow-up testing using available clinical methods is required, if the HEPACARD test is non-reactive with persisting clinical symptoms."

End of Report

Dr.Seema Pavan
MBBS, MD(Pathology),PGDMLE
Consultant Pathologist, KMC N0. 36984

MC- 6085

PACKAGE TRACK SHEET

Package : : CNH-HPHC-03

Name	: Mr. NIRANJANA DATTA K A / 47 Year(s) / Male	**Date**	: 15/03/2025 9:02AM
UHID No.	: RXML.44153	**Bill No.**	: SVCR3071
Start Time	: 09:02:19	**Completion Time**	:
Corporate Company	:	**Payer**	: CONNECT AND HEAL PRIMARY CARE PVT ~~LTD~~
Insurance	:		

Time In	Time Out	Signature/Date Time

Initial Assessment

Height. **Weight.** **BMI.**

Pulse. **BP.** **Temp.**

Package Details

1. Healthcheck

25-Hydroxy Vitamin D Total-()
Blood Urea Nitrogen (BUN)-()
Haemogram-()
C-Reactive Protein-()
Glycated Haemoglobin (Hba1C)-()
Hbs Ag (Rapid Card Test)-()
LFT - Liver Function Test-()
Lipid Profile-()
Peripheral Smear-()
PSA (Prostatic Specific Antigen Total)-()
Serum Calcium-()
Serum Creatinine-()
Serum Uric Acid-()
TSH-Thyroid Stimulating Hormone-()
Complete Urine Analysis-()
FBS-Fasting Blood Sugar-()
USG Abdomen and Pelvis-() (P) Evening after
X-Ray Chest PA-() Refused
TMT OR ECHO-()
Electrocardiogram (ECG)-()
Physical Examination-()
Pulmonary Function Test (PFT)-()

Start Time	End Time	Remarks
—	—	Refused.
—	—	Bhavya
—	—	Bhavya
—	—	
—	—	Bhavya

2. OP Consultation

General Medicine-(Mr. ML-HC)

Start Time	End Time	Remarks

11:10am.

Dental Surgery

901 409 1111
901 996 1402
(Chat with our rep on WhatsApp)
rxdx.in | info@rxdx.in

Name:	Niranjana Datta K A	**UHID:**	
Age / DOB:	47 / Sun, Aug 7, 1977	**Sex:**	None
Email:	ndatta@hpe.com	**Your Location:**	Bengaluru
Date:	Sat, Mar 15, 2025	**Branch:**	RxDx SAMANVAY, Malleswaram

Patient Vitals Section

Q38. Height (cm)	176.
Q39. Weight (kg)	75.01kg.
Q40. BMI	
Q41. Waist (cm)	100'
Q42. Hip (cm)	96
Q43. Chest (cm)	95
Q44. Inhalation	98
Q45. Exhalation	95
Q46. Pulse	76b/m 98%
Q47. BP	120/80.
Q48. Near Vision: Right	N16
Q49. Near Vision: Left	N/6.
Q50. Distance Vision: Right	D/6
Q51. Distance Vision: Left	D/6.

Vital Signs™ 226 166 05

CE 0537 LOT D790

Niranjana
ID: rxml.44153

Male

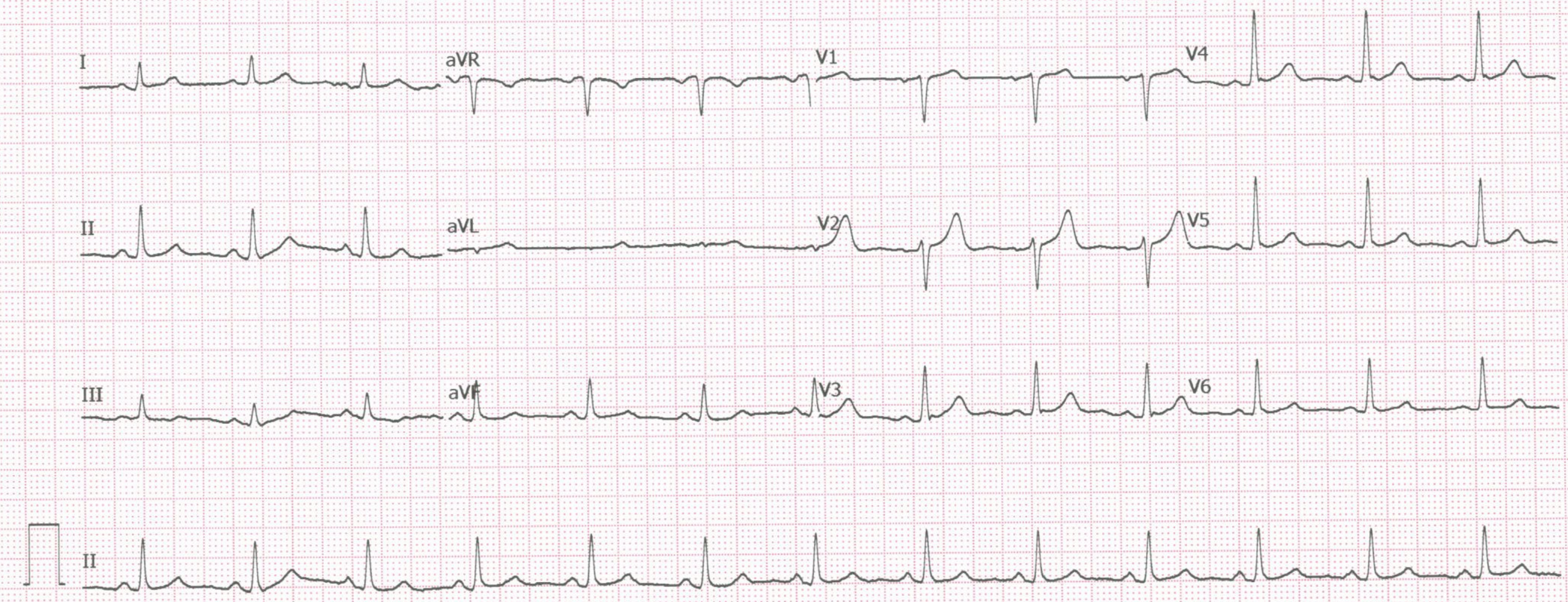
47 Years

15.03.2025 9:26:44
RXDX SAMANVAY
29, 11th Main, Malleshwaram
Bangalore

79 bpm
-- / -- mmHg

QRS :	82 ms
QT / QTcBaz :	362 / 415 ms
PR :	140 ms
P :	96 ms
RR / PP :	754 / 759 ms
P / QRS / T :	62 / 56 / 39 degrees

Normal sinus rhythm
Normal ECG



Unconfirmed

CE MAC2000 1.1 12SL™ v241 25 mm/s 10 mm/mV ADS 0.56-20 Hz 50 Hz 4x2.5x3_25_R1 1/1

RxDx

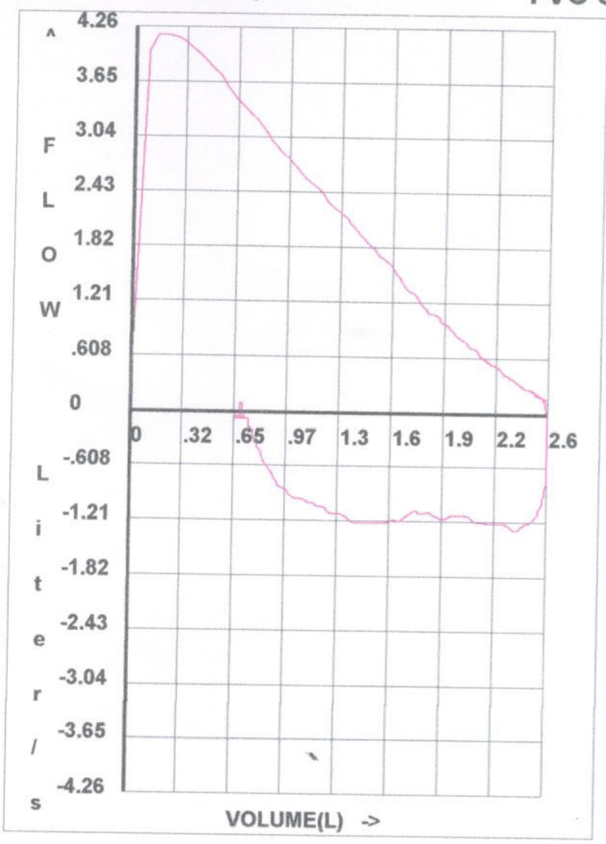
Samanvay

PULMONARY FUNCTION TEST REPORT

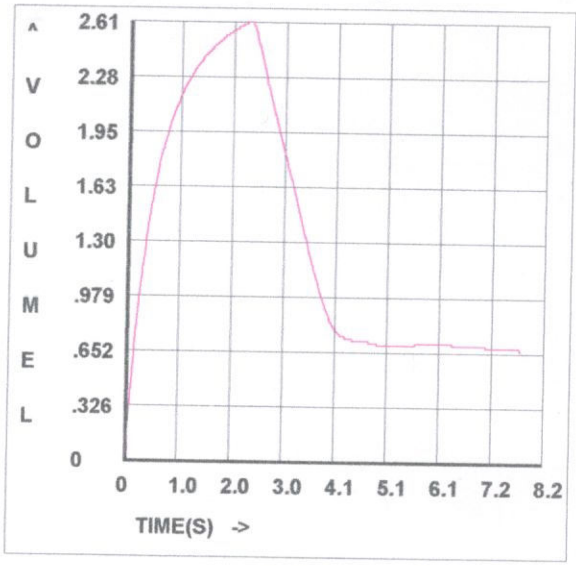
ID No. : 1185
Patient Name : NIRANJANA
Age(yrs.) : 47 Sex: M
Report Date : 15-03-2025
Eth. Corr.(%) : 80
Weight(Kg): 75
Height(cm): 176
BSA(m^2): 1.91
Indications :
Comments :
History of Smoking : Non - Smoker

FVC GRAPHS

Flow / Volume Graph



Volume / Time Graph



TEST RESULTS

Date of Test ->	15/03/2025		
Time of Test ->	09:40:32		
Parameter (U)	Pred.#	Actual	%Pred.
FVC (L)	3.66	2.611	71.33
FEV0.5 (L)		1.572	
FEV1 (L)	2.972	2.158	72.61
FEV1/FVC %	78.75	82.65	104.9
PEF (L/s)	7.148	4.263	59.63
PIF (L/s)		1.332	
FEF25-75% (L/s)	3.275	2.041	62.32
Vmax25% (L/s)	6.221	3.73	59.95
Vmax50% (L/s)	3.891	2.309	59.34
Vmax75% (L/s)	1.625	1.066	65.6
FET100% (s)		2.42	

EST. Lung Age (Yrs.) 82

INTERPRETATION: Mild Res Mild Obs ESA Obs Low VC

This may be clinically co - related.

#Predicted Equations : ERS93

RXDX SAMANVAY
MALLESWARAM
BANGALORE

Date :

EXERCISE STRESS TEST REPORT

Patient Name: DATTA K A, NIRANJANA
Patient ID: RXML.44153
Height: 176 cm
Weight: 75 kg

DOB: 07.08.1977
Age: 47yrs
Gender: Male
Race: Indian

Study Date: 15.03.2025
Test Type: Treadmill Stress Test
Protocol: BRUCE

Referring Physician: DR RAGHUNANDN BK
Attending Physician: DR RAGHUNANDAN BK
Technician: BHAVYA

Medications:
none, none, none, none, none, none, none, none, none, none
Medical History:
NO

Reason for Exercise Test:
HEALTH CHECK

Exercise Test Summary

Phase Name	Stage Name	Time in Stage	Speed (mph)	Grade (%)	HR (bpm)	BP (mmHg)	Comment
PRETEST	SUPINE	00:04	0.00	0.00	102	120/80	
	STANDING	00:04	0.00	0.00	101	120/80	
	HYPERV.	00:04	0.00	0.00	96	120/80	
	WARM-UP	00:05	0.00	0.00	92	120/80	
EXERCISE	STAGE 1	03:00	1.70	10.00	127	120/80	
	STAGE 2	03:00	2.50	12.00	144	130/80	
	STAGE 3	03:00	3.40	14.00	162	140/80	
	STAGE 4	00:01	3.40	14.00	162		
RECOVERY		03:42	0.00	0.00	104	140/80	

The patient exercised according to the BRUCE for 9:00 min:s, achieving a work level of Max. METS: 10.10. The resting heart rate of 104 bpm rose to a maximal heart rate of 171 bpm. This value represents 98 % of the maximal, age-predicted heart rate. The resting blood pressure of 120/80 mmHg , rose to a maximum blood pressure of 140/80 mmHg. The exercise test was stopped due to THIRD STAGE COMPLETE.

Interpretation

Summary: Resting ECG: normal.
Functional Capacity: normal.
HR Response to Exercise: appropriate.
BP Response to Exercise: normal resting BP - appropriate response.
Chest Pain: none.
Arrhythmias: none.
ST Changes: none.
Overall impression: Normal stress test.

Conclusions

GOOD EFFORT TOLERANCE
NORMAL HR/BP RESPONSE
NO ANGINA/ARRHYMIAS
NO SIGNIFIANT ST-T CHANGES
***TMT NEGATIVE FOR INDUCIBLE ISCHEMIA**